



JDScience BTEC Health and Social Care Unit 2 Set C mark scheme

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Use this booklet after the matching practice paper. Do not issue it with the student paper.

Time: 1 hour 30 minutes · **Total marks:** 80

Question 1 · 20 marks

Westbridge Hospice (voluntary sector) works with an NHS palliative care consultant, district nurses and a local authority carer's officer. Families can also buy extra night sits from a private agency. A faith group provides volunteer sitting.

(a) [2 marks]

Identify two types of care shown in this case (for example primary, secondary, tertiary, informal).

- Tertiary/specialist palliative (consultant/hospice); primary (district nurses); informal (faith group); private additional care. Award any two correctly labelled.

(b) [4 marks]

Describe the purpose of hospice care.

- Holistic support for life-limiting illness: pain/symptom control, emotional, social and spiritual support, family support, not only end-of-life beds.

(c) [6 marks]

Explain why multi-agency working is important in palliative care.

- Needs cross health, social care and spiritual support; 24-hour cover; carer breakdown prevention; consistent plans (DNACPR, preferred place of death) communicated.

(d) [8 marks]

Discuss funding and access issues that families may face in this mix of provision.

- NHS/hospice charitable funding vs paying for night sits — inequality.
- Eligibility for local authority carer support; waiting times; rural transport (if applied carefully — case does not specify rural).
- Level 3: discussion of mixed economy of care with a conclusion about fairness/access.

Question 2 · 20 marks

Ben is 28 and has a spinal cord injury. He uses a wheelchair. He sees a physiotherapist, an occupational therapist, a specialist nurse and a personal assistant funded through a direct payment. The GP surgery has steps at the entrance and a bell for assistance.

(a) [2 marks]

Identify two allied health professionals in this case.

- Physiotherapist; occupational therapist.

(b) [4 marks]

Describe how an occupational therapist could support Ben's independence.

- Home/work adaptations, equipment, energy conservation, practising daily living tasks, advising the PA and GP surgery on access.

(c) [6 marks]

Explain two physical barriers Ben may face and how they could be reduced.

- Surgery steps: ramp, accessible entrance, not relying only on a bell.
- Transport, narrow doors, inaccessible toilets — reasonable adjustments under the Equality Act.

(d) [8 marks]

Evaluate the use of direct payments for people with physical disabilities.

- Control over who supports him and when; person-centred.
- Responsibility to employ/manage a PA; risk if unwell; need for information and support to use the payment well.
- Level 3: balanced evaluation applied to Ben.

Question 3 · 20 marks

A school nurse is supporting Year 10 sessions on consent, confidentiality and where to get sexual health advice. A pupil asks the nurse not to tell anyone they are attending a clinic. The nurse also has a safeguarding duty.

(a) [2 marks]

Identify two principles of the 6Cs or care values relevant to this work.

- Care, compassion, competence, communication, courage, commitment — or confidentiality, respect, safeguarding. Award two.

(b) [4 marks]

Outline when a school nurse may keep a young person's information confidential.

- If the young person is competent (Gillick/Fraser principles in sexual health contexts), information is kept private unless there is risk of significant harm or exploitation.

(c) [6 marks]

Explain how the nurse should balance confidentiality with safeguarding.

- Explain limits of confidentiality at the start; keep routine clinic attendance private; share if abuse, trafficking, or serious risk; share only with those who need to know; record decisions.

(d) [8 marks]

Discuss why trust matters when young people use health services.

- Without trust they may not seek contraception/STI care → worse public health and individual harm.
- Trust is built by honesty about what will be shared.
- Safeguarding still protects those who cannot consent or are at risk.
- Level 3: discussion with a justified conclusion.

Question 4 · 20 marks

An inspector finds that a day service for adults with learning disabilities has no up-to-date DBS checks for two staff, incomplete accident records, and that activities are the same every week regardless of individual plans. The manager says they are "short-staffed".

(a) [2 marks]

Identify two laws or types of regulation that apply to this service.

- Examples: Health and Social Care Act / CQC regulation; DBS/safeguarding requirements; Equality Act; Health and Safety; Data Protection. Award two.

(b) [4 marks]

Describe two possible consequences of poor record-keeping.

- Injuries not followed up; cannot evidence care; inspection failure; risk in emergencies; families lose trust.

(c) [6 marks]

Explain how person-centred planning should influence daily activities.

- Individual goals, interests and communication needs — not one timetable for all.
- Short-staffing is not a reason to ignore plans; must escalate and risk-assess, not quietly drop personalisation.

(d) [8 marks]

Evaluate the manager's responsibilities for safer recruitment and staffing.

- DBS and references before unsupervised work; rotas that allow safe care; reporting vacancies to the provider/local authority if quality is at risk.
- “Short-staffed” explains pressure but does not excuse illegal/unsafe practice.
- Level 3: clear judgement on accountability.

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